

Patient ID	: MM02287621	Patient Name	: Mrs. Sandhya Malik
Gender	: Female	Age	: 50Y
Encounter ID	: 20582352	Encounter Type	: Inpatient
Admission Date	: 30/09/2023 00:08	Location	: 11th Floor B1
Speciality	: Gastroenterology	Attending Practitioner	: Dr Rajesh Puri

Patients Address : G-56, AVENUE-23, BEHIN WARKING WOMENS HOSTEL, SAKET, Saket
S.O, SOUTH DELHI, India, 110017

Date of Discharge : 04/10/2023 11:19

Bed No : 4150

Discharge Status : Stable at Discharge

Diagnosis & Co-morbidities

:
Well differentiated Neuroendocrine tumor (G1) , ki 67 <1 % with multiple hepatic mets
Intracerebral hemorrhage (October 2021)
Menopause (November 2021)

Advice on Discharge

Discharge Medication :

Tablet CEFTUM 500mg twice daily for 5 days

Tablet TRYPTOMER 25mg once at night

Tablet PANTOCID 40mg once daily (before breakfast)

Capsule A TO Z 1 capsule once daily

Tablet ULTRACET 1 tablet twice daily for 3 days followed by as and when required for pain

Tablet ONDEM-MD 4mg as and when required for vomiting / nausea

VOLINI GEL for local application twice daily

Diet Normal diet

Follow up

:
Review with Gastroenterology OPD with Dr. Rajesh Puri / Dr. Zubin Dev Sharma on 01.11.2023 (Wednesday) with Complete Blood Count, Liver Function Test, Renal Function Test & Prothrombin Time / International Normalized Ratio reports.
(before coming contact (before coming contact Ms. Sukhjeet Kaur @ 9999411125)

Follow up in Medical Oncology Outpatient Department (OPD) with CBC with diff , LFT , RFT after 4 weeks for next dose (1st dose of Injection Sandostatin LAR 30 mg deep IM given on 03.10.2023)

Medical History & Presenting Complaints

:
Mrs. Sandhya Malik, 50 Years old female, follow up case of Hypertension, Well differentiated Neuroendocrine tumor (G1) , ki 67 <1 % , presented with complaints of abdominal pain since 4-5 days, right upper quadrant, radiating to back, localized to right hypochondria. She is admitted here for further evaluation and management.

Physical & Systemic Examination

:
On admission patient was conscious, oriented
Saturation of Peripheral Oxygen (SPO2): 98% on room air
Temperature: 98°F
Pulse: 78/min
Respiratory rate: 20/min
Blood pressure: 110/80mmHg
Chest: Bilateral air entry present

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Per abdomen: Soft, tenderness +
Central Nervous System : No focal neurological deficit
Cardiovascular System: S1 & S2 normal

Course in Hospital

:

Patient was admitted with above mentioned complaints for which relevant investigations were done. She was started on Intravenous fluids, Intravenous antibiotics along with supportive measures.

Ultrasound Whole Abdomen was done on 29.09.2023 which showed grade-I fatty changes. Few ill-defined hypoechoic lesions are seen measuring upto 2.6 x 1.3cm. Gall bladder partially distended and shows a calculus of size 11.7mm in its lumen.

Computed Tomography (CT) Whole Abdomen was done on 01.10.2023 which showed metastatic neuroendocrine tumor, Previously seen segmental enhancing wall thickening in the distal jejunal / proximal ileal loop in the right paracentral abdomen is still evident and it measures up to 6 mm in thickness involving up to 4 to 5 cm of small bowel length. Associated mesenteric soft tissue enhancing mass like deposit. Slight increased trans-mural enhancement and subtle concentric thickening is noted in few of the mid jejunal loops which measure up to 5 mm in thickness. Liver is mildly enlarged in size and measures approximately 21 cm in cranio-caudal extent, shows smooth margins with non-cirrhotic features. There are multiple variable sized intra-hepatic metastatic peripherally enhancing hypodense lesions seen in both lobes of liver which show no serial significant interval change in comparison to DOTA NOC PET CT dated 21/09/2023. Cholelithiasis.

Medical Oncology opinion was taken in view of Well differentiated Neuroendocrine tumor and advice to start Inj Sandostatin LAR 30 mg deep im every 4 weekly.

1st dose of Injection Sandostatin LAR 30 mg deep IM given on 03.10.2023

Diet was upgraded as tolerated. During hospital stay, she was managed with Intravenous antibiotics, Intravenous fluids and other supportive measures. Now patient is being discharged with follow up advice and medication.

Procedure/Surgery

INVESTIGATIONS

	Date	Date	Date	Date
Hemoglobin (gm/dl)	(29.09.2023) 12.3 gm/dl			
TLC (10³/uL)	10.84 10 ³ /uL			
Platelets (10⁵/uL)	291 10 ³ /uL			
ESR				
CRP				
B.Urea / Serum Creatinine (mg/dl)	16 mg/dl 0.70 mg/dl			
Na/K (mEq/L)	141 mmol/L / 4.5 mmol/L			
Bilirubin T/D (mg/dl)	0.47 mg/dl 0.33 mg/dl			
AST/ALT (U/L)	21 U/L 18 U/L			
Total Protein/Albumin (gm/dl)	7.81 gm/dl / 3.96 gm/dl			
SAP/GGT (U/L)	88 U/L / < 10 U/L			
PT/INR				

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MELD Na

: Lipase - 94.00 U/L
Amylase - 89 U/L

Radiology

: Attached

Consultant Incharge**WHEN TO OBTAIN URGENT CARE:**

In case of any problem like:-

1. Fever/Jaundice
2. Loose stools/motions/vomiting/vomit contain blood or passing black stools like coal tar.
3. Bleeding from any site
4. Chest pain, breathing difficulty, profuse sweating, giddiness, pain in abdomen.
5. Reduced urine output.
6. Severe weakness/severe mouth ulcers.
7. Rash over skin, swelling over body.
8. Unconscious

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FOR EMERGENCY AND AMBULANCE PLEASE CONTACT: 1068.

Summary prepared by (MT): : .

Consultant Dr Zubin Dev Sharma

Phone No

Signature

Sr. Resident Dr. Harshal

Phone No 9834207638

Signature

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- . Sample Collection
- . Nursing Visits
- . Medicine Delivery
- . Child and Adult Vaccination

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